



THE PLAN

The APPG on Black Maternal Health

What to say and do at the All-Party Parliamentary Group, Committee Room
10, 22 September, 3:00pm.

Status labels apply throughout: KNOWN, VERIFIED, ASSUMPTION, ESTIMATE, TO VALIDATE, PROPOSED. Nothing in this document claims traction, approvals, partners or revenue that do not exist. Governing file: FACTS_BASE.

PREPARED FOR

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What to say and do at the All-Party Parliamentary Group, Committee Room 10, 22 September, 3:00pm.

In one minute

- This is not a selling room. Sell in it and you will be remembered for the wrong reason.
- Go to listen. Say little. Ask one good question. Follow up with three people the next day.
- Never say technology fixes this. The national reports say the cause is racism and not being listened to.
- The one honest thing your software does: it writes down what a woman said and when. That matches what families say was missing.
- Do not hand out the old prediction deck. Do not claim any approval, partner or pilot. Everyone in that room can check.

What to do

Action	Who	By when
Arrive at Cromwell Green by 2:15pm with photo ID and the invitation on your phone	You	Today, 2:15pm
Listen for the first hour. Do not speak until the open floor	You	Today
Ask your one question, and give your name and one line only	You	Today
Get three names and emails in the break, offer nothing but help	You	Today
Send three follow-up emails, each offering something, asking for nothing	You	Within 24 hours

Read the room before you open your mouth

An All-Party Parliamentary Group is an informal cross-party group of MPs and peers. It is not an official committee and it has no power to buy anything. Its value to you is the people in the room, not the group itself.

Who will be there: bereaved families, campaigners, midwives, obstetricians, trust maternity leads, researchers, charity staff and parliamentary researchers. Many of them have lost someone or have sat with people who have. Several will know the evidence far better than you do.

This means three things. Your statistics are their statistics, so do not recite them back.

Your product is not the subject of the meeting. And anyone who appears to be there to sell will be quietly noted and not invited again.

What is live right now, so you sound current

What	When	Why it matters to you
Baroness Amos, National Maternity and Neonatal Investigation	Published 30 June 2026	Eight recommendations. Found racism, discrimination, and families not being listened to. Proposes a Maternity and Neonatal Commissioner answering to Parliament
MBRRACE-UK, Maternal mortality 2022-2024	8 January 2026	Black women at nearly three times the risk. 252 deaths reviewed. Better care might have changed the outcome in 61% of them
Health and Social Care Committee, Black Maternal Health	2025	Named racism as one of the core drivers. Asked for mandatory cultural competency training
The maternal morbidity indicator system	Government committed, going live	A national data system on maternal harm. This is the one to understand properly

That last row is the most useful line in this document. The government has committed to building a national data system about maternal harm. That is a data problem, and data problems are what you are actually good at. Understand it before you pitch anything to anyone.

The position that works, and the one that ends you

Do not say: that an app will close the gap, that prediction will save these women, that better technology is the answer, or that your system spots deterioration earlier. The evidence does not say the problem is detection. It says women raised concerns and were not believed.

Do say, if and only if someone asks you directly:

"I do not think software fixes racism. What I have seen in the reports is that women say they raised something and nobody acted, and afterwards there is no record of it. The one honest thing software can do is write down what she said, when she said it, and what happened next. That makes it harder to say later that she never mentioned it."

That claim is small, it is true, and it is the only version of your product that this room will respect. It also happens to be the version that survives scrutiny from a regulator.

Your twenty-second answer to "what do you do"

"I am Kelvin. I am building maternity software called MaternaLink. It is in development, nothing is live, and I have no customers yet. I came to listen, because I would rather understand this properly than build the wrong thing."

Then stop talking and ask them what they do. The people who get remembered well at these events are the ones who asked good questions, not the ones who described themselves at length.

Your one question, if there is an open floor

Ask about the thing you genuinely need to know. Something like:

"On the maternal morbidity indicator system the department has committed to: will it capture what a woman reported and when, or only clinical outcomes after the event? And will the women themselves be able to see what was recorded about them?"

That question is useful to the room, it is not about you, and it tells every serious person present that you understand where this is going. Give your name and say only that you build maternity software. Nothing else.

Things that will damage you

1. **Handing out the 2025 prediction deck.** It claims the software predicts pre-eclampsia, sepsis, bleeding and clots. There is no evidence for that and it is withdrawn. Do not carry it.

1. **Any claim of approval or partnership.** You have no company, no certification, no pilot and no NHS relationship. Saying otherwise in this room is the end of your credibility.

1. **Posting afterwards that you have partnered with the APPG or with Parliament.** You attended a meeting. That is all. Never imply endorsement, and never use the parliamentary portcullis emblem on anything you produce.

1. **Working the room with business cards during the testimony.** Wait for the break.

2. **Photographing or quoting attendees.** Families tell painful stories at these events.

Do not film, do not photograph people, do not repeat anything with a name attached.

1. **Using the mortality figure as a sales line.** Not in that room, not on LinkedIn afterwards.

Who to find in the break, in order

Who	Why	What to say
The MP's parliamentary researcher or the group's secretariat	They control who gets invited back and what goes on the agenda	"Is there a mailing list? I would like to keep following this properly"
Any midwife, obstetrician or trust maternity lead	This is your clinical safety officer, your design partner and your first site	"Can I send you something and get five minutes of your view on whether it is useful?"
Charity and campaign staff	They know every trust, every commissioner and every funder	Ask what they wish existed. Write the answer down
Researchers and academics	Evidence partners, and the route into grant money	"Who is doing the data work on this?"

Do not queue to speak to the MP. Every other person in the room is trying to. The researcher standing behind them is more useful to you and is free.

The follow-up email, sent within 24 hours

Keep it to five lines. Offer something. Ask for nothing.

Subject: Yesterday's APPG — the point you made about records Dear [Name], We met briefly at the Black Maternal Health group yesterday. What you said about [the specific thing they said] stayed with me. I am building maternity software and I am at the stage where I would rather be told I am wrong than find out later. If it is ever useful, I would gladly show you what I have and take the criticism. No reply needed if you are busy. Thank you for what you said yesterday. Kelvin Williams

The person who writes that gets a reply. The person who attaches a pitch deck does not.

Practical

Cromwell Green visitors' entrance. Allow thirty minutes for security. Bring photo identification and the invitation. No smoking or e-cigarettes, no parking. Dress as you would for a hospital meeting. Take a notebook and write things down by hand; it reads as respect.

Words explained

Word	What it means
APPG	An informal group of MPs and peers on one subject. It has no legal power and buys nothing
Secretariat	The organisation that runs an APPG day to day, often a charity or campaign group
MBRRACE-UK	The UK's national investigation into why mothers and babies die, run from Oxford
Maternal morbidity	Serious harm to a mother that did not kill her. Much more common than death, and less well recorded
Design partner	An early clinical partner who gives you real time and honest criticism before you have a product